

Office of Enforcement and Removal Operations
ICE Health Service Corps

U.S. Department of Homeland Security
500 12th Street, SW
Washington, D.C. 20536



U.S. Immigration
and Customs
Enforcement

November 27, 2023

MEMORANDUM FOR:

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Assistant Director
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

THROUGH:

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CDR (b)(6),(b)(7)(C),(b)(7)(F) RN
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FROM:

LCDR (b)(6),(b)(7)(C),(b)(7)(F) RN
Investigator, Investigations Unit
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

SUBJECT:

Mortality Review – Report of Findings
Julio Cesar Chirino PERALTA, (b)(6),(b)(7)(C)

Executive Summary:

On October 8, 2023, U.S. Immigration and Customs Enforcement (ICE) Health Service Corps (IHSC) received notification of the death of noncitizen Julio Cesar Chirino PERALTA, (b)(6),(b)(7)(C). ICE assigned Mr. PERALTA, a 32-year-old Nicaraguan male, to Port Isabel Detention Center (PIDC), in Los Fresnos, Texas (TX), from September 12 to October 8, 2023, the date of his death. On October 23, 2023, a Valley Baptist Medical Center (VBMC) physician certified Mr. PERALTA's cause of death as subarachnoid hemorrhage (bleeding in the space between the brain and the thin tissues covering the brain) secondary to acute myelogenous leukemia (cancer of the blood and bone marrow) and noted his manner of death as natural.

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On September 10, 2023, the United States Border Patrol (USBP) arrested Mr. PERALTA, near Brownsville, TX, upon his illegal entry into the United States and charged him with violating Section 212 (a)(7)(A)(i)(I) of the Immigration and Nationality Act (INA), as a noncitizen present in the United States without an immigrant visa or other valid entry document. On September 12, 2023, USBP transferred Mr. PERALTA to ICE Enforcement and Removal Operations (ERO) Harlingen, and ERO detained Mr. PERALTA at PIDC.

On September 12, 2023, a PIDC registered nurse (RN) completed Mr. PERALTA's intake screening and documented his denial of any current or past medical histories, except abdominal cramps, and cleared him for general population (GP).

Between September 19 and October 3, 2023, the PIDC's health and dental staff provided Mr. PERALTA treatment for his periodontitis (gum infection with damages to soft tissue around teeth) diagnosis and general weakness and headache.

On September 20, 2023, an RN completed Mr. PERALTA's initial physical exam, documented a normal exam and Mr. PERALTA's denial of any past medical or mental health histories, and cleared him for GP.

On October 5, 2023, at approximately 7:45 a.m., Mr. PERALTA experienced left side paralysis (loss of movement or control), and a PIDC custody officer announced the medical emergency via radio. Medical staff responded to the emergency, and at approximately 7:54 a.m., PIDC activated emergency medical services (EMS). EMS personnel transported Mr. PERALTA to the VBMC emergency room (ER), in Harlingen, TX, where a physician admitted him with a diagnosis of suspected acute leukemia (blood cancer) with disseminated intravascular coagulation (a condition in which blood clots form throughout the body).

- On the same date, an ER physician intubated (a procedure involving the passage of a tube into the trachea to help a person breathe) Mr. PERALTA, admitted him to the neurological intensive care unit on ventilatory support, and ordered intravenous critical care medications.

Between October 5 and 6, 2023, Mr. PERALTA remained intubated and continued receiving critical care medications, antibiotics, mechanical ventilatory support, cryoprecipitate (blood plasma), platelets, and hydroxyurea (cancer medicine).

On October 8, 2023, at 7:46 p.m., a VBMC physician declared Mr. PERALTA deceased.

Mortality Finding:

As a result of Mr. PERALTA's death, the IHSC Assistant Director requested a mortality review to learn from his death by reviewing the care provided and the circumstances leading up to his

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death. The goal of the mortality review is to determine the appropriateness of clinical care; ascertain whether changes to policies, procedures or practices are warranted; and identify issues requiring further study.

A mortality review committee (MRC) determined the care PIDC provided Mr. PERALTA deviated beyond safe limits of practice, specifically, the MRC concluded: 1) PIDC dental staff's documentation conflicted with their plan of care; 2) PIDC administered a medication that was contraindicated; 3) PIDC's custody officer responded with an expired BLS certification; 4) the MRT tasked with calling EMS did not immediately know the nature of Mr. PERALTA's emergency, which delayed the call; and 5) a PIDC RN did not appropriately refer Mr. PERALTA to a higher level of care.

The MRC convened on November 8, 2023, and consisted of the following IHSC members:

Captain (CAPT) (b)(6),(b)(7)(C),(b)(7)(F) deputy medical director, physician reviewer; CAPT (b)(6),(b)(7)(C),(b)(7)(F) chief, advanced practice provider (APP), APP reviewer; CAPT (b)(6),(b)(7)(C),(b)(7)(F) chief nurse, nurse reviewer; and CAPT (b)(6),(b)(7)(C),(b)(7)(F) chief dentist.

The following summary of events was based on: 1) Mr. PERALTA's PIDC medical records; 2) Mr. PERALTA's EMS report; 3) Mr. PERALTA's death certificate; 4) incident and notification reports; 5) ICE ENFORCE Alien Removal Module (EARM) and ICE ENFORCE Alien Detention Module (EADM) database records; 6) Mr. PERALTA's PIDC detention file; 7) video footage dated October 5, 2023; 8) an on-site review and staff interviews conducted by Lieutenant Commander (LCDR) (b)(6),(b)(7)(C),(b)(7)(F) and LCDR (b)(6),(b)(7)(C),(b)(7)(F) at PIDC on November 2, 2023; and 9) applicable IHSC policies and procedures, and ICE Detention Standards.

Summary of Events

On September 12, 2023, at 8:30 p.m., a PIDC RN reviewed Mr. PERALTA's USBP transfer summary (no medical issues noted), completed his pre-screening, and documented his denial of any current or past medical or mental health conditions.

- At 11:45 p.m., an RN completed Mr. PERALTA's intake screening, documented his denial of any current or past medical conditions, except abdominal cramps, and cleared him for GP.

On September 19, 2023, during sick call, an RN evaluated Mr. PERALTA for right lower molar pain for two days (worsened with brushing) and documented normal vital signs, mild swelling around the gum, and no abscess. Using a clinical nurse guideline (CNG) for dental pain, the RN provided Mr. PERALTA with ibuprofen (pain medicine), 200 milligrams (mg) tablet, by mouth (PO), as needed (PRN) for one day, and instructed him on proper oral hygiene.

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On September 20, 2023, an RN evaluated Mr. PERALTA for complaints of moderate right lower posterior molar pain for the past three to four days. The RN documented normal vital signs, visible edema noted to right cheek and surrounding gum areas to posterior right lower molar and no abscess. Using a CNG for dental pain, the RN provided Mr. PERALTA ibuprofen 200 mg for one day and referred him to dental for a higher level of care.

- At 1:55 p.m., a dentist evaluated Mr. PERALTA for dental pain and documented right buccal mucosa swelling caused by tooth #32 biting the cheek, a mild eruption of tooth #32, and poor oral hygiene. The dentist diagnosed Mr. PERALTA with swelling and infection of the gum tissue, ordered oral antiseptic twice daily for 14 days, antibiotics, three times daily (TID) for seven days, and ibuprofen 600 mg with food, PO, TID, PRN for 14 days.
- At 3:36 p.m., an RN completed Mr. PERALTA's initial physical exam, documented a normal exam and Mr. PERALTA's denial of any past medical or mental health histories, and cleared him for GP.

On September 22, 2023, during a dental follow-up evaluation, Mr. PERALTA reported some improvement in his dental pain. The dentist documented the inability to obtain an x-ray due to closure of dental clinic, "no additional medications required at this time," and cleared Mr. PERALTA for GP.

On September 26, 2023, a registered dental hygienist (RDH) completed Mr. PERALTA's dental follow-up evaluation, documented the inability to obtain an x-ray due to the closure of the dental clinic, mild gum swelling, and Mr. PERALTA's self-reported improvement of his gum swelling and pain. The RDH instructed Mr. PERALTA to return to sick call if his condition worsened.

On September 27, 2023, Mr. PERALTA submitted a sick call request for dental pain.

- At 1:51 p.m., an RN evaluated him and documented normal vital signs, no signs of infection, carious top molars, and bottom molar pain. Using a CNG for dental pain, the RN provided Mr. PERALTA acetaminophen (pain medicine) 325 mg, two tablets, PO, instructed him to return to medical if pain worsens, and cleared him for GP.

On September 29, 2023, a dentist completed Mr. PERALTA's follow-up evaluation, documented his completion of antibiotics four days prior, mild fever resolved with acetaminophen, nocturnal dental pain to (right) lingual mucosa swelling and redness adjacent to tooth #32, a grossly decayed tooth #1, and persisting periodontitis. The dentist noted the inability to obtain a dental x-ray due to "clinic closure and repair," ordered amoxicillin-potassium clavulanate (antibiotic), 875/125 mg tablet, PO, every 12 hours for seven days, and instructed Mr. PERALTA to return for further evaluation should his condition worsen.

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On September 30, 2023, during urgent sick call, an RN evaluated Mr. PERALTA for complaints of general weakness and headaches for two days. The RN documented Mr. PERALTA's self-reported complaint of fatigue and muscle aches, his ongoing dental treatment for pericoronitis (swelling and infection of the gum tissue) with antibiotics, and normal vital signs. The RN instructed Mr. PERALTA on proper handwashing, encouraged him to drink two to three liters of water daily, and to return to medical if he develops chest pain, numbness/tingling, nausea and vomiting, visual disturbances, or any unrelieved headache, and cleared him for GP.

On October 3, 2023, during a dental follow-up evaluation, Mr. PERALTA reported worsening dental pain radiating to his right ear and affecting his ability to chew and eat. The dentist documented no improvement in symptoms despite two rounds of antibiotics, normal vital signs, and exam, except a two-pound decrease in Mr. PERALTA's body weight. The dentist ordered a soft diet and referred Mr. PERALTA to an oral surgeon for extraction of tooth #1 and #32.

- On the same date, an oral surgeon evaluated Mr. PERALTA and documented a periapical abscess around tooth #32 and a non-restorable fracture of teeth #1, #3, and #14 requiring surgical extraction. The surgeon shared his findings with Mr. PERALTA, who requested immediate extraction of the affected teeth.
- Upon Mr. PERALTA's return to the facility, an RN completed his off-site return assessment, documented normal vital signs, and cleared him for GP. Additionally, the dentist noted per the oral surgeon, Mr. PERALTA requires a second visit to perform extractions within one week of consultation.

On October 5, 2023, at 7:35 a.m., Mr. PERALTA reported left sided weakness and pain and inability to move and requested medical assistance.

- At 7:38 a.m., custody staff contacted the medical clinic and requested assessment of Mr. PERALTA's medical problem.
- At 7:43 a.m., two health staff arrived at Mr. PERALTA's dorm and advised the dormitory officer to call a code 2 (medical emergency).
- At 7:45a.m., custody staff announced a (b)(7)(E) via handheld radio.
- At approximately 7:48 a.m., additional responding health staff arrived at Mr. PERALTA's dormitory. An APP noted Mr. PERALTA was lying on the top bunk with his airway and circulation intact, but Mr. PERALTA self-reported he had been unable to move the left side of his body for the past three hours. The APP documented responding health staff transferred Mr. PERALTA from the bunk onto a stretcher.

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- At approximately 7:52 a.m., responding health staff departed the dormitory with Mr. PERALTA and transported him to the medical building.
- At approximately 7:53 a.m., health staff arrived at the medical building and proceeded to an urgent care room, where they established a right arm peripheral line, administered a cellular fluid replacement solution, oxygen, and four baby aspirins. An APP documented normal vital signs, except increased blood pressures (BP) readings of 147/93 and 125/85 milliliters of mercury [(mmHg); normal range 90/60 – 120/80 mmHg], and a heart rate of 105 beats per minute [(bpm); normal range 60 – 99 bpm].
- At 7:54 a.m., a medical records technician called 911 and requested an ambulance for an individual having a stroke at the facility.
- At 7:55 a.m., the 911 operator dispatched paramedics to PIDC.
- At 8:11 a.m., paramedics arrived at PIDC.
- At 8:15 a.m., paramedics arrived at the scene and assumed Mr. PERALTA's care.
- At 8:30 a.m., paramedics departed PIDC and transported Mr. PERALTA to VBMC's ER for a higher level of care.
- At 9:01 a.m., paramedics arrived at VBMC's ER.
- At 9:10 a.m., a VBMC ER physician assumed Mr. PERALTA's care.
- At 10:15 a.m., a VBMC ER physician documented Mr. PERALTA presented with left-sided paralysis and bleeding in the mouth. The physician also noted Mr. PERALTA scored a 14 (mild to moderately severe stroke symptoms) on the National Institute of Health (NIH) stroke scale (15-item severity scoring - 0: no stroke, 1-4: minor, 5-15: moderate, 16-20: moderate to severe, and 21-42: severe). A computed tomography scan of Mr. PERALTA's head showed a subdural hematoma (brain bleed), and his laboratory studies showed an elevated white blood cell count and international normalized ratio (test for determining how long it takes blood to clot), and a low platelet count of 37,000 platelets per microliter (normal: 150,000 to 450,000). The ER physician documented suspected acute leukemia with disseminated intravascular coagulation and consulted oncology, neurosurgery, and an intensivist.
- Hours later, the ER physician intubated Mr. PERALTA, admitted him to the neurological intensive care unit on ventilatory support, and ordered intravenous critical care drips of mannitol (treats brain swelling), nicardipine (high blood pressure medicine), levetiracetam (seizure medicine), fentanyl (pain medicine), propofol (sedative-hypnotic),

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midazolam (sedative), pantoprazole (anti-acid), corticosteroids, and antibiotics (piperacillin/tazobactam and vancomycin).

Between October 5 and 6, 2023, Mr. PERALTA remained intubated and continued receiving critical care drips (propofol, fentanyl and midazolam), antibiotics, mechanical ventilatory support, cryoprecipitate, platelets (blood cells that prevent clotting), and hydroxyurea.

On October 7, 2023, at approximately 12 p.m., VBMC health staff documented Mr. PERALTA's NIH score increased to 30., dilated pupils unreactive to light, and low blood pressure managed with norepinephrine (treatment for low blood pressure).

On October 8, 2023, at 7:46 p.m., a VBMC physician declared Mr. PERALTA deceased.

ICE detention standards used for this review: Performance-Based National Detention Standards (PBNDS), 2011 with 2016 Revisions, and National Commission on Correctional Health Care (NCCHC) Standards for Health Services in Jails, 2018. The following are identified program strengths, weaknesses, and recommendations found during this review.

PIDC Strengths

Strengths identified during this investigation were: 1) PIDC's health and custody staff worked cohesively during Mr. PERALTA's medical emergency; 2) PIDC's nursing staff provided Mr. PERALTA timely medical care using the clinical nursing guidelines during sick call; 3) PIDC's dental staff collaborate with health staff by providing a written plan for health staff to follow when providing urgent and emergent dental services; 4) PIDC's health staff responded to the medical emergency with the proper equipment (oxygen, stretcher, emergency bag, and an automated external defibrillator (machine used to treat cardiac arrest); 5) PIDC's health staff responded to the medical emergency within four minutes; 6) PIDC's health staff promptly referred Mr. PERALTA to VBMC via ambulance for a higher level of care.

Weaknesses

1. Documentation

- On September 20, 22, 26, and 29, 2023, PIDC dental staff documented the inability to obtain in-house x-rays due to closure of the dental clinic and did not pursue an off-site referral until October 3, 2023.

Applicable standards of care for this finding:

- NCCHC: Standards for Health Services in Jails, 2018: J-E-06, *Oral Care*.
- NCCHC: Standards for Health Services in Jails, 2018: J-D-03, *Clinic Space, Equipment, and Supplies*.
- NCCHC: Standards for Health Services in Jails, 2018: J-E-09, *Continuity Coordination, And Quality of Care During Incarceration*.

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- NCCHC: Standards for Health Services in Jails, 2018: J-D-08, *Hospital and Specialty Care*.
- NCCHC: Standards for Health Services in Jails, 2018: J-A-01, *Access to Care*.
- IHSC Directive: 09-01-G-01, Dental Service Guide, 2023; section H. *Referral/Specialty Care (Off-Site Dental Care)*.

2. Patient Safety

- On October 5, 2023, an APP instructed nursing staff to administer four baby aspirins (blood thinner) to Mr. PERALTA during an active hemorrhagic stroke (bleeding inside the brain) and before confirming whether Mr. PERALTA had hemorrhagic vs. ischemic stroke (cutting off blood supply to the brain).

Applicable standards of care for this finding:

- NCCHC: Standards for Health Services in Jails, 2018: J-B-08, *Patient Safety*.
- NCCHC: Standards for Health Services in Jails, 2018: J-D-02, *Medication Services*.
- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section G. *Pharmaceutical Management*.

3. Emergency Response

- All written incident reports reviewed were missing information about events leading up to the activation of (b)(7)(E).
- PIDC's custody staff did not direct other noncitizens away from Mr. PERALTA's bunk section to provide adequate scene safety.
- One PIDC custody staff member who participated during Mr. PERALTA's medical emergency response had an expired basic life support card.

Applicable standards of care for this finding

- PBNDS 2011 with 2016 Revisions: Part 1.1, Emergency Plans; section II. *Expected Outcomes*.
- PBNDS 2011 with 2016 Revision: Part 4.3, Medical Care; section T. *Emergency Medical Service and First Aid*.

4. Communication

- The MRT tasked with calling 911 did not immediately know the nature of the 911 call. After receiving the information, the MRT provided the information to the 911 operator (approximately 4 minutes after PIDC'S APP informed custody staff to call 911).

Applicable standards of care for this finding

- PBNDS 2011 with 2016 Revision: Part 4.3, Medical Care; section T. *Emergency Medical Service and First Aid*.

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5. Referral

- On September 30, 2023, the RN, who completed Mr. PERALTA's sick call encounter for general weakness, deviated from IHSC sick call policy by not referring Mr. PERALTA to a provider for a higher level of care.

Applicable standards of care for this finding

- IHSC Directives: 03-02 G-01, Sick Call Guide, 2021; section II. *Sick Call Process*

Recommendations

- Forward these findings to the IHSC Deputy Assistant Director (DAD) of Health Care Compliance (HCC).
- The IHSC DAD of HCC will share these findings through appropriate communication channels to ICE, PIDC's administrator and health authority for review and any action(s) deemed appropriate.
- The respective IHSC HCC unit and ICE will ensure the corrective action plan, if applicable, is implemented and sustained.

End of report.